



Restore

AI that wins dental claims back.

The thing I'm building, in plain English — and how you can be part of it.

FOUNDER

Ross Richardson

STAGE

Pre-seed · pilot launching

LIVE DEMO

`restore-demo.vercel.app`

I started this because of my mom.

Mom runs a 4-doctor endodontic practice. Endodontists are the dental specialists who do root canals. The work is technical, the cases are big, and the insurance side is brutal.

Carriers deny **20-30% of valid claims** on first pass. Mom's office manager spends 90 minutes drafting a single appeal letter. When you do the math — \$40/hr labor, 50% chance of winning — most appeals don't even break even. So practices write the money off. **\$500K-\$2M a year, gone.**

I spent 7 years running financial operations at Method Co. (10 hotels, restaurants, ~\$200M revenue). I'm an operator, not a healthcare insider. That's the unfair advantage: I see the workflow, not the medicine.

I'm building the tool I'd want if I were running mom's office. An AI that watches the practice's software, writes the insurance correspondence, and queues it up for a one-click approval.

♡ DESIGN PARTNER

Allyson A. Abbott DMD PC

Mom's practice is the design partner. She tells me in 30 seconds whether what I built actually works. She's not a cheerleader — she's switched dental software vendors three times in ten years when products didn't deliver.

The pilot kicks off next month.

Insurance is a math problem the practice loses.

The reason claims get written off isn't that the practice doesn't care. It's that fighting insurance is a labor-cost game the practice can't win.

90 min

to write one appeal letter

Office manager has to know each carrier's narrative preferences. Cigna wants anatomical specificity. Delta wants exact CDT terminology. MetLife wants conservative-alternatives language.

35-50%

success rate without carrier intel

A practice-drafted appeal wins about half the time. Combined with the 90 minutes of labor, the expected value of a single appeal is barely break-even.

\$500K-\$2M

written off per practice / year

That's revenue the practice earned, treatment they delivered. The carrier said "denied," the office manager said "I don't have 90 minutes," and the money disappeared.

Generic AI text generators don't fix this either. Each carrier scores narratives differently. **Generic appeals lose 60-70% of the time.** What you need is per-carrier intelligence — and that's what Restore is.

An AI agent that lives inside the practice's software.

It does three things — three documents the office manager would otherwise write by hand. They're the highest-volume, highest-friction parts of the day.

Pre-authorization narratives

15-25 / WEEK

Before a procedure happens, the carrier needs a clinical narrative explaining why it's medically necessary. Restore drafts it from the chart, tuned to the specific carrier's preferences.

Claim appeal letters

8-15 / WEEK

When a claim gets denied, Restore reads the EOB, identifies the denial pattern, and drafts the appeal. Sometimes Restore says 'don't appeal — this is a contractual exclusion.' That discipline is unique.

Referral letters

80-160 / WEEK

Endodontists get most patients from general dentists. After treatment, the office sends a letter back to the referring GP. Restore drafts it in collegial-clinical voice.



The agent watches for events in the practice management system (PBS Endo, the software mom uses). When something happens — a treatment plan finalized, an EOB landing, a procedure completed — Restore drafts the right document and queues it for review.

Watch. Draft. Approve. Submit.

The whole thing takes **60-90 seconds** per item. The same work used to take **90 minutes**. That's a 60x compression — and the office manager is still in control of every output.

01

Watch

Restore listens to the practice's software for triggers. New treatment plan? EOB landed with a denial code? Treatment marked complete? Restore sees it the moment it happens.

02

Draft

Restore reads the case context — chart notes, prior history, what we know about that specific carrier's preferences — and drafts the appropriate document. It also tells the office manager why it's confident: '92% historical approval rate on this evidence pattern.'

03

Approve

Office manager opens the inbox, reviews the draft (60-90 seconds), and either approves it, edits it, or rejects with a reason. The office manager is always in the loop. Nothing goes out without human sign-off.

04

Submit

Once approved, Restore submits to the insurance clearinghouse + saves a copy to the practice's document center. Audit trail captured. The whole arc is logged for the carrier intelligence layer to learn from.

The office manager keeps full control. The AI does the typing.

That distinction is critical — for compliance, for trust, and for the practice owner who's accountable for what gets sent.

The whole thing is built. You can see it now.

The demo at restore-demo.vercel.app is the actual product. Real workflows, real interactions, real carrier intelligence. Mock data only for the patient side (since the real pilot hasn't started yet).

INBOX — THE DAILY WORKFLOW

PRE-AUTH D3348 retreatment · Tooth #14 · Cigna DPPO

\$1,450 HIGH

REFERRAL Dr. Marcus Patel · Sunset Family Dentistry

— HIGH

APPEAL D3331 obstruction · Tooth #3 · Delta Dental

\$385 HIGH

PRE-AUTH D3425 apicoectomy · Tooth #30 · MetLife

\$1,675 MED

Each item expands to show the full draft + carrier-specific reasoning. Approve / edit / reject — done in one tap.

THE 90-DAY INSURANCE LIFECYCLE

Most "AI for dental" tools generate one document and forget it. Restore threads the whole arc together so nothing falls through:

- 1 **Pre-auth drafted + submitted** — by Restore
- 2 **Carrier approves or denies** — tracked
- 3 **Treatment happens** — claim auto-submitted
- 4 **EOB lands** — if denied → appeal drafted
- 5 **Appeal submitted** — by Restore (or 'DO NOT APPEAL')
- 6 **Resolution** — case closed, money recovered or written off

Restore gets smarter every week.

The product isn't the AI — anyone can call an AI. The product is the per-carrier playbook that compounds with every claim that flows through.

01

Each carrier has quirks

Cigna wants anatomical specificity. Delta wants exact CDT terminology. MetLife wants conservative-alternatives-considered framing. UnitedHealthcare's bar on sedation is the highest in the industry. We've already mapped these.

02

We learn what wins and what loses

Every claim that gets approved or denied feeds back. After enough volume, we know exactly which narrative pattern Cigna approves on D3348 retreatments. After more volume, we know which sub-pattern wins at 92% vs. 67%.

03

1 practice's lesson → 1,000 practices' edge

When mom's practice discovers that Cigna is bundling D3331 codes more aggressively than usual, every practice on Restore knows by tomorrow. That network effect is what no individual practice (and no generic AI) can replicate.

 A new entrant who tries to compete on day one has zero carrier data. Even with the same AI tech, they'd start at **35-50% appeal win rate** while Restore is at **73%+ and climbing**. Catching up takes years of paying customers — that's the moat.

The product works. The pilot starts next month.

TODAY (MAY 2026)

Built & live

- ✓ Full prototype shipped at [restore-demo.vercel.app](#)
- ✓ 3 skills working: pre-auth, appeals, referrals
- ✓ Carrier playbook for top 6 carriers
- ✓ Mom's practice signed as design partner
- ✓ Anthropic BAA executed (HIPAA cover)

NEXT 90 DAYS

Pilot live

- Pilot live at mom's 4-doc practice
- First non-mom paying customer
- Quantified pilot metrics (success rate vs. baseline)
- 3 LOIs in active conversation

NEXT 12 MONTHS

Real revenue

- 25 paying practices
- ~\$750K annual recurring revenue
- First DSO conversation in late stage
- Institutional seed round (Q3 2026)

The market is real (~5,500 endodontic practices in the US, expanding to 30,000+ specialty + 178,000 GP + DSO consolidation). But we're not chasing the whole market on day one. **We're winning endo first because the math works hardest there.**

\$250K from friends & family. SAFE. 6-month bridge.

Friends-and-family rounds exist because the people who know you best want to back you before the institutional capital arrives. This is the cheapest price the company will ever raise at.

ROUND STRUCTURE

Round size	\$250K target
Instrument	Post-money SAFE
Valuation cap	\$5M post-money
Discount	20%
Minimum check	\$10K
Maximum check	\$50K (preserves seed for institutional)
Founder commitment	Full-time at seed close · \$150K personal cash

SAFE converts at the institutional seed round. F&F gets a 50% discount to that round's \$10M cap (so \$5M effective price), plus an additional 20% conversion discount. You're paying ~half what the seed investors will pay.

WHAT IT FUNDS (NEXT 6 MONTHS)

- **Pilot scale-up** — engineering contractor for PMS hardening, deeper carrier playbook
- **Customer acquisition** — first 5 paid customers via study clubs + referrals
- **Compliance setup** — HIPAA audit, BAA legal, SOC 2 scoping
- **Founder transition** — bridge cost while resigning Method Co. role

WHY THIS ROUND, WHY NOW

The institutional seed conversation is Q3 2026. Between now and then I need 6 months of runway to get from "1 design partner" to "5 paying customers." That milestone is what unlocks the seed round at a meaningfully higher valuation. F&F money is the bridge.

A short, honest ask.

If you want to invest, talk to me. **Minimum \$10K, maximum \$50K**, post-money SAFE at a \$5M cap. Documents and process will be clean, simple, and lawyer-reviewed.

If you can't or don't want to invest — that's totally fine. **Please refer me to the next 5 endodontists or specialty practice owners you know.** The practices we close in the next 90 days determine the institutional valuation.

Either way, I appreciate you being here. Mom appreciates you being here. The endodontists who'll get paid faster because of this product appreciate you being here.

FOUNDER

Ross Richardson

GET IN TOUCH

✉ richardson112288@gmail.com
restore-demo.vercel.app

